



HEALTH INSURANCE CLAIM FORM

APPROVED BY NATIONAL UNIFORM CLAIM COMMITTEE 02/12

PROGRESSIVE MEDICAL CLAIMS

P.O. BOX 22031 (PIP)

ALBANY, NY 12201

1. medicare ☐ (medicare #) ☐ medicare ☐ (medicaid #) ☐ tricare ☐ ID#/DoD# ☐ champva ☐ (Member ID#) ☐ group health plan (SSN or ID) ☐ fecla blk lung (SSN) ☐ other ☒ (id)		1a. insured's i.d. number (for program in item 1) 113732639A081264	
2. PATIENT'S NAME (Last Name, First Name, Middle Initial) CRYSTAL WILLIAMS		3. PATIENT'S BIRTH DATE MM DD YY 09 08 1984 SEX M ☐ F ☒	
5. PATIENT'S ADDRESS (No., Street) 561 Barbridge		6. PATIENT RELATIONSHIP TO INSURED Self ☐ Spouse ☐ Child ☐ Other ☐	
CITY Brooklyn		STATE NY	
ZIP CODE 11233		TELEPHONE (Include Area Code) (347) 707-4263	
9. OTHER INSURED'S NAME (Last Name, First Name, Middle Initial)		10. IS PATIENT'S CONDITION RELATED TO:	
a. OTHER INSURED'S POLICY OR GROUP NUMBER		a. EMPLOYMENT? (Current or Previous) ☐ YES ☒ NO	
b. RESERVED FOR NUCC USE		b. AUTO ACCIDENT? ☒ YES ☐ NO	
c. RESERVED FOR NUCC USE		c. OTHER ACCIDENT? ☐ YES ☒ NO	
d. INSURANCE PLAN NAME OR PROGRAM NAME		10d. CLAIM CODES (Designated by NUCC)	
11. INSURED'S POLICY GROUP OR FECA NUMBER 07675056-0			
a. INSURED'S DATE OF BIRTH MM DD YY SEX M ☐ F ☐			
b. OTHER CLAIM ID (Designated by NUCC)			
c. INSURANCE PLAN NAME OR PROGRAM NAME PROGRESSIVE MEDICAL CLAIMS			
d. IS THERE ANOTHER HEALTH BENEFIT PLAN? ☐ YES ☒ NO If yes, return to and complete item 9a-d			
12. PATIENT'S OR AUTHORIZED PERSON'S SIGNATURE I authorize the release of any medical or other information necessary to process this claim. I also request payment of government benefits either to myself or to the party who accepts assignment below. SIGNED SIGNATURE ON FILE DATE 10/31/2011			
13. INSURED'S OR AUTHORIZED PERSON'S SIGNATURE I authorize payment of medical benefits to the undersigned physician or supplier for services described below. SIGNED SIGNATURE ON FILE			
14. DATE OF CURRENT ILLNESS, INJURY, or PREGNANCY (LMP) MM DD YY 07 15 2011 QUAL.		15. OTHER DATE QUAL. MM DD YY	
17. NAME OF REFERRING PROVIDER OR OTHER SOURCE Dr. Azu Ajudua		17a. ☐ 17b. NPI ☐	
19. ADDITIONAL CLAIM INFORMATION (Designated by NUCC)			
21. DIAGNOSIS OR NATURE OF ILLNESS OR INJURY (Relate Items 1, 2, 3 or 4 to Item 24E by Line) ICD Ind 9 1. 723.1 2. 737.42 3. 728.85 4. 724.2 5. 737.30 6. 722.93 7. 722.6 8. 729.4			
22. RESUBMISSION CODE ORIGINAL REF. NO. 23. PRIOR AUTHORIZATION NUMBER			
24. A. DATE(S) OF SERVICE From To B. PLACE OF SERVICE C. EMG D. PROCEDURES, SERVICES OR SUPPLIES (Explain Unusual Circumstances) E. DIAGNOSIS POINTER F. \$ CHARGES G. DAYS OR UNITS H. EPSDT Family Plan I. ID. QUAL. J. RENDERING PROVIDER ID. #			
09 29 11 09 29 11 11 72141 1 2 3 4 5 6 7 8 \$879.73 1 N NPI 183333			
10 05 11 10 05 11 11 72148 1 2 3 4 5 6 7 8 \$912.00 1 N NPI 183333			
10 05 11 10 05 11 11 73721 1 2 3 4 5 6 7 8 \$878.67 1 N NPI 183333			
10 11 11 10 11 11 11 73221 1 2 3 4 5 6 7 8 \$878.67 1 N NPI 183333			
25. FEDERAL TAX I.D. NUMBER SSN EIN 26-1323136 ☐ ☒		26. PATIENT'S ACCOUNT NO. 41	
27. ACCEPT ASSIGNMENT? (For govt claims, see back) ☐ YES ☒ NO		28. TOTAL CHARGE \$3,549.07	
29. AMOUNT PAID \$		29. AMOUNT PAID \$3,549.07	
30. Rsvd for NUCC			
31. SIGNATURE OF PHYSICIAN OR SUPPLIER INCLUDING DEGREES OR CREDENTIALS (I certify that the statement on the reverse apply to this bill and are made a part thereof) WINTER W STEVEN W. M.D. SIGNED DATE 10/31/2011		32. SERVICE FACILITY LOCATION INFORMATION STELTON RADIOLOGY CORP 1849 Utica Avenue Brooklyn, NY 11234	
33. BILLING PROVIDER INFO & PHONE # STELTON RADIOLOGY CORP 1849 Utica Avenue Brooklyn, NY 11234 (718) 676-5595			



STEVEN W. WINTER, M.D. P.C.

1849 Utica Avenue Brooklyn, NY, 11234
Phone:(718) 676-5595 Fax:(718) 709-7499

PATIENT NAME: Crystal Williams
DATE OF SERVICE: 09/29/2011
REFERRING PHYSICIAN: Azu Ajudua

MRI Cervical spine:

Multi-planar, multi-echo images through the cervical spine were performed.

There is straightening of the cervical lordosis.

There is no evidence of discogenic disease, disc bulge or disc herniation. There is no compromise of the cervical cord, thecal sac or neural foramina. There is no bony destructive lesion or paravertebral soft tissue mass.

- 1. There is no cervical disc herniation.**
- 2. Straightening of the cervical lordosis.**

Thank you for the courtesy of this consultation.

Winter Steven W., M.D.

Diplomate American Board of Radiology Board Of Education 1234567890.

Diplomate American Board of Radiology.



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1849 Utica Avenue Brooklyn, NY, 11234
Phone:(718) 676-5595 Fax:(718) 709-7499

PATIENT NAME: Crystal Williams

DATE OF SERVICE: 10/05/2011

REFERRING PHYSICIAN: Azu Ajudua

MRI Left Knee:

Multi-planar, multi-echo images through the left knee were performed.

Both the medial collateral ligament and anterior cruciate ligament is inhomogeneous with ligamentous sprains.

There is partial extension of the body of the medial meniscus outside the medial joint compartment with medial compartment narrowing. There is a component of lateral patellofemoral subluxation and there is a trace amount of synovial fluid at the patellofemoral articulation.

Otherwise the cruciate and collateral ligaments and both menisci show no other regions of signal alteration or morphological abnormality. The articular surfaces and bone marrow signal of the knee are otherwise without morphologic or signal alterations, and the remainder of the soft tissues surrounding the knee are otherwise intact.

- 1. Both the medial collateral ligament and anterior cruciate ligament is inhomogeneous with ligamentous sprains.**
- 2. There is partial extension of the body of the medial meniscus outside the medial joint compartment with medial compartment narrowing. There is a component of lateral patellofemoral subluxation and there is a trace amount of synovial fluid at the patellofemoral articulation.**

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PATIENT NAME: Crystal Williams
DATE OF SERVICE: 10/05/2011
REFERRING PHYSICIAN: Azu Ajudua

MRI Lumbar spine:

Multi-planar, multi-echo images through the lumbar spine were performed. There is reduction of image clarity due to the patient's body habitus. The best obtainable study was performed.

There is a right lumbar convexity.

At the L4/5 level there is a posterior disc bulge extending into the epidural fat that nearly abuts the thecal sac. There is a peripheral component of the disc bulging encroaching into the neural foramen slightly more so on the left than the right.

There is diminished T1 and increased T2 signal intensity noted in the posterior paraspinal facial tissues, compatible with inflammatory/non-infectious posterior paraspinal fascitis.

Otherwise, there is no evidence of discogenic disease, disc bulge, disc herniation or compromise of the thecal sac, lateral recesses or neural foramina. There is no conus medullaris lesion, fracture, bony destructive lesion or paravertebral soft tissue mass otherwise demonstrated.

1. There is a right lumbar convexity.
2. At the L4/5 level there is a posterior disc bulge extending into the epidural fat that nearly abuts the thecal sac. There is a peripheral component of the disc bulging encroaching into the neural foramen slightly more so on the left than the right.
3. Diminished T1 and increased T2 signal intensity noted in the posterior paraspinal facial tissues, compatible with inflammatory/non-infectious posterior paraspinal fascitis.

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1849 Utica Avenue Brooklyn, NY, 11234
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PATIENT NAME: Crystal Williams
DATE OF SERVICE: 10/11/2011
REFERRING PHYSICIAN: Azu Ajudua

MRI Left Shoulder:

Multi-planar, multi-echo images through the left shoulder were performed. The patient was not able to remain optimally still for the examination and there is reduction of image clarity. The best obtainable study was performed.

Despite motion artifact, there appears to be a bulbous morphology to the supraspinatus tendon, indicating tendinosis. Additionally, there is an inferiorly oriented acromioclavicular joint which abuts the underlying rotator cuff at the musculotendinous junction.

The patient has a mesoacromion which can be a noteworthy finding in the setting of rotator cuff impingement.

The examination otherwise demonstrates the muscular and tendonous structures of the shoulder including the remaining portions of the rotator cuff to demonstrate no morphologic or signal abnormalities. The bone marrow signal of the shoulder otherwise shows no signal alteration or deformity. The glenohumeral joint and glenoid labrum are otherwise unremarkable in position and signal characteristics.

- 1. Bulbous morphology to the supraspinatus tendon, indicating tendinosis. Additionally, there is an inferiorly oriented acromioclavicular joint which abuts the underlying rotator cuff at the musculotendinous junction.**
- 2. The patient has a mesoacromion which can be a noteworthy finding in the setting of rotator cuff impingement.**
- 3. Images demonstrate reduction of clarity due to motion artifact.**

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